



ICD-O-3

Carcinoma, Urothelial NOS
8120/3

Gender: Female

PathSite: Bladder, Anterior Wall,
Posterior Wall and Dome
C67.8

Race: Black

CcCF: Bladder, lateral wall
C67.2

Pathology Report:

QW12/24/12

Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

A. Urinary bladder, uterus, cervix, bilateral fallopian tubes, and bilateral ovaries; pelvic exenteration:

- Invasive high grade urothelial carcinoma with squamous differentiation $< 10\%$ per TSS, invading into the bladder muscularis propria, see pathologic parameters.
- Uterus with inactive endometrium.
- Cervix with no diagnostic abnormality.
- Bilateral atrophic ovaries.
- Bilateral fallopian tubes, with no diagnostic abnormality.

B. Pelvic lymph nodes, right; dissection:

- Twelve lymph nodes, no tumor (0/12).

C. Pelvic lymph nodes, left; dissection:

- Eleven lymph nodes, no tumor (0/11).

D. Common iliac lymph nodes, left; dissection:

- Seven lymph nodes, no tumor (0/7).

E. Common iliac lymph nodes, right; dissection:

- Ten lymph nodes, no tumor (0/10).

F. Presacral lymph nodes; dissection:

- Seven lymph nodes, no tumor (0/7).

G. Distal ureter, left; excision:

- Portion of ureter, no tumor.

H. Distal ureter, right; excision:

- Portion of ureter, no tumor.

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive urothelial carcinoma with squamous differentiation.
2. Grade of tumor: High grade.
3. Depth of invasion: Muscularis propria, outer half.
4. Tumor distribution: Multifocal, 4.5 cm, at bladder left lateral, posterior, and dome walls; second ulceration at right superior-anterior bladder wall.
5. Ureteral margins: Negative for tumor.
6. Distal urethral margin: Negative for tumor.
7. Soft tissue margin or serosa: Negative for tumor.
8. Lymph nodes: Negative for tumor (0/47).
9. pTNM: pT2b,N0,MX.

Effective _____ this Checklist utilizes the 7th edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

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[], MD

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [], M.D.

Electronically Signed Out by [], MD

Clinical History:

Patient is a _____ year-old female with urothelial cancer undergoing a pelvic exenteration.

Specimens Received:

A: Uterus, cervix, bilateral fallopian tubes, bilateral ovaries, urethra, bladder

B: Right pelvic lymph node

C: Left pelvic lymph node

D: Left common iliac lymph node

E: Right common iliac lymph node

F: Presacral lymph node

G: Left distal ureter

H: Right distal ureter

Gross Description:

The specimens are received in eight containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "uterus, cervix, bilateral tubes and ovaries, urethra, bladder". Received fresh and placed in formalin is a 236.0 gm, 13.5 x 11 x 5 cm pelvic exenteration specimen composed of a bladder, uterus with complete adnexa, cervix and vaginal mucosa. The

bladder is 8.5 x 6 x 3 cm, tan-pink with a smooth glistening serosal surface. On both the posterior wall and left lateral aspect of the bladder dome serosa there is a 2.5 x 1 cm area of slightly firm contracted bladder wall. The left ureter is 2.5 x 0.4 cm and the right ureter is 4.5 x 0.5 cm and each is probe patent. Attached at the inferior aspect is a segment of urethra that is 4.3 cm in length by 1.5 cm in diameter. The contracted serosal surface and the lateral resection margins are inked black. The bladder is opened along the lateral walls to reveal an ulcerated granular tan-pink to red lesion (4.5 cm in length by 1.2-2.5 cm in width) extending from the posterior wall into the dome on the left lateral half of the bladder. The lesion appears to invade into the submucosa and into the posterior wall on the left lateral aspect of the bladder. There is a second ulcerated contracted tan-pink granular lesion on the mucosa of the right superior-anterior wall measuring 1.2 cm in length by 1 cm in width. Between the two ureteral orifices, superior to the trigone on the posterior wall is a 0.7 x 0.4 x 0.4 cm focus of transparent raised nodular tissue. The bladder is serially sectioned to reveal a scaly indurated tan-white cut surface with multifocal red-brown hemorrhage with attached soft yellow glistening adipose tissue. The lesion appears diffusely distributed across the cut surface of the anterior wall, posterior wall and bladder dome. The first lesion appears to invade to a maximum depth of 1.4 cm and is close to the black inked serosa near the bladder dome. The second contracted lesion appears to invade to a depth of 1.1 cm. The bladder wall ranges in thickness from 0.9-2.1 cm. The remaining uninvolved bladder mucosa is tan-pink wrinkled and hyperemic. The perivesicular adipose tissue is dissected for lymph nodes and no lymph nodes are identified.

Also received as part of the specimen is a uterus with attached adnexa measuring 4 cm from fundus to cervix by 4 cm from cornu to cornu by 2.7 cm from anterior to posterior. The anterior serosal surface is tan-pink smooth glistening and grossly unremarkable. The posterior serosal surface is tan-pink dull and matted but otherwise unremarkable. The right ovary is tan-white cribriform measuring 3 x 1.1 x 1 cm and the right fimbriated fallopian tube is 8.5 x 0.5 cm tan-pink, slightly indurated and adhered to the ovary. The left ovary is tan-white, cribriform measuring 2.5 x 1.5 x 0.8 cm with 2 firm white nodules measuring 0.4 cm in greatest dimension and a left fimbriated fallopian tube that is 7 x 0.4 cm tan-pink rubbery and adhered to the ovary. The cervix is 3.5 cm in length. The cervical face is ill-defined, tan-pink and roughened measuring approximately 2.5 x 2 cm. The cervical os is stenotic. The uterus is opened along the right lateral wall to reveal a 2 x 0.7 cm, glistening, tan endocervical canal tan-pink and glistening leading into a triangular endometrial cavity measuring 3 x 2.7 cm. The endometrium is tan-pink glistening with an average thickness of 0.1-0.2 cm. The myometrium has a well-circumscribed tan palpably firm area measuring 0.7 x 0.7 x 0.3 cm on the superior anterior uterine wall. The remaining myometrium is tan, trabeculated and grossly unremarkable. Attached inferior to the uterus is a segment of pink smooth glistening vaginal mucosa measuring 9.3 x 6.5 cm.

Gross photographs are taken. Representative sections are submitted as follows:

A1: Urethral margin
A2: Right ureter margin
A3: Left ureter margin
A4-A5: First lesion on bladder dome and posterior wall, closest approach to black ink
A6-A7: Second lesion on superior-anterior wall
A8: Small raised transparent nodular lesion superior to the trigone
A9-A10: Representative sections of uninvolved bladder mucosa
A11-A12: Representative sections of anterior endomyometrium (includes palpably firm area) and cervix
A13-A14: Representative sections of posterior endomyometrium and cervix
A15: Representative sections of left ovary and fallopian tube
A16: Representative sections of right ovary and fallopian tube

B. The second container is additionally identified as, "2. Right pelvic lymph node". Received fresh and placed in formalin is a 5.5 x 5 x 2 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal 12 apparent lymph nodes ranging from 0.6-3.2 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

B1: 4 lymph nodes
B2: 3 lymph nodes
B3-B7: One bisected lymph node in each cassette

C. The third container is additionally identified as, "left pelvic lymph node". Received fresh and placed in formalin is a 5.2 x 4.5 x 1.3 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal 16 apparent lymph nodes ranging from 0.3-2.2 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

C1-C2: 3 lymph nodes in each cassette
C3: 4 lymph nodes
C4: 2 lymph nodes
C5-C8: One bisected lymph node in each cassette

D. The fourth container is additionally identified as, "4. left common iliac lymph node". Received fresh and placed in formalin is a 4.5 x 4.2 x 1.2 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal 7 apparent lymph nodes ranging from 0.2-2.2 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

D1: 3 lymph nodes
D2: 2 lymph nodes

D3-D4: One bisected lymph node in each cassette

E. The fifth container is additionally identified as, "5. Right common iliac lymph nodes". Received fresh and placed in formalin is a 3.8 x 2.3 x 1 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal 10 apparent lymph nodes ranging from 0.5-1.8 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

E1-E3: 3 lymph nodes in each cassette

E4: One bisected lymph node

F. The sixth container is additionally identified as, "6. Presacral lymph node". Received fresh and placed in formalin is a 3.7 x 2.5 x 1 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal 8 apparent lymph nodes ranging from 0.2-1.8 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

F1: 2 lymph nodes

F2: 3 lymph nodes

F3: 2 lymph nodes

F4: One bisected lymph node

G. The seventh container is additionally identified as, "left distal ureter". Received fresh and placed in formalin is a 1.2 cm in length by 0.2 cm in diameter segment of tan-white tubular tissue. Attached at one end is a long double stitch which designates the non-margin side as stated on the accompanying requisition. The non-margin side is inked blue and the margin is inked black. The specimen is bisected and submitted entirely in cassette G1.

H. The eighth container is additionally identified as, "right distal ureter". Received fresh and placed in formalin is a 2.1 cm in length by 0.3 cm in diameter segment of tan-white tubular tissue. Attached at one end is a long double stitch which designates the non-margin side as stated on the accompanying requisition. The non-margin side is inked blue and the margin is inked black. The specimen is trisected and submitted entirely in cassette H1.

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Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 12/20/12	